

Transfer of Care to Provider From Another Agency

Standard:

To define circumstances and establish a process for receiving and transferring patients from one agency to another.

Purpose:

Providers may be presented with multiple patients, limited resources, or patient conditions requiring transfer of care to a provider from another agency, such as: transferring care to an aeromedical provider, a different transporting agency, another agency that will obtain a patient refusal, etc.

The ultimate decision of whether or not to transfer care to a provider from another agency shall be made following all established norms, Texas laws, and the Clinical Standard regarding [On-Scene Authority of Patient Care](#). During Mass and Multi-casualty incident situations, decisions will be made by the on-scene command structure.

Application:

When transferring care to a provider of a different agency:

- This decision must, above all, be patient-centered. Transferring care to another agency must benefit the patient and, if possible, the EMS system.
- Patient care officially transfers from one provider to the other after an in-person report, which should, at minimum, consist of the following, in MIST format:
 - **Differential Diagnosis**
 - Age/Sex
 - Mechanism of Injury / Medical Complaint
 - Injuries / Inspections – time of onset, brief medical exam & findings
 - Vital Signs – First set & significant changes
 - Treatments and times
- Providers are encouraged and expected to continue to assist in patient care within their scope of practice as needed by other agencies. This can include providing care during transport, should the transporting agency express the need. The transporting agency's lead provider has authority over patient care and other providers should assist them in their plan of care. If a significant disagreement arises, OLMC should be contacted.
- Complete all documentation following the [Documentation of Patient Care Report](#) Clinical Standard.
- When a system-credentialed physician (PL-8) or advanced provider (PL-7) is on-scene, providers should make allowances for them to inform clinical care across agencies.